NCCP 00947 · VERSION 1

Capivasertib + fulvestrant

Capivasertib is taken twice daily for four days, then stopped for three days, each week. Fulvestrant injections are given on Days 1 and 15 of the first 28-day cycle, then Day 1 of later cycles. Treatment continues while helpful and tolerated.

Make treatment fit into daily life

The usual starting capivasertib dose is 400 mg twice daily, about 12 hours apart, on the prescribed dosing days. Swallow tablets whole, with or without food; avoid grapefruit. Your prescription may use a lower dose. Each usual 500 mg fulvestrant dose is two slow injections into muscle, one in each buttock; no IV port or pump is required.

Injection visits and extra tests

Two fulvestrant injection visits in the first cycle, then one every 28 days. Glucose and other monitoring can create extra checks between injection visits.

Tablets and supply

Capivasertib tablet strengths are 160 mg and 200 mg. The team confirms the correct number and strength for the prescribed dose; retain the recurring four-day/three-day calendar.

Access and practical planning

No IV port, infusion or take-home pump is required by this two-medicine regimen. Additional treatments may have separate arrangements.

Missed doses and pauses

If a capivasertib dose is late, it may be taken within 4 hours of its usual time, leaving at least 8 hours before the next dose. If more than 4 hours late, skip it and take the next dose at the usual time. Do not double a dose. If you vomit after a dose, do not take an extra dose; take the next scheduled dose at its usual time. Contact the team if vomiting continues or prevents fluids or medicines.

Your support contacts

Agree daytime and out-of-hours contacts, access to glucose testing, and what to do if a test, prescription or injection appointment is missing.

Who arranges each check?

Agree the due window, booking team, result reviewer and whom to contact if an appointment or result is missing. This reference page does not book or record a test.

Sources and review

The NCCP regimen supplies the treatment rules. Separate guidelines supply the longer-term monitoring recommendations, with their qualifications retained.

- NCCP 00947 v1 · official regimen
- EMA Truqap product information
- EMA Faslodex product information
- NCCP oral antiemetic guidance v6
- Turner NC et al. Capivasertib in Hormone Receptor-Positive Advanced Breast Cancer. NEJM 2023;388:2058–2070.
- Rugo HS et al. Final overall survival results from phase III CAPItello-291. ESMO Breast 2026, abstract 417O. ESMO Open 11(suppl 4): 107440.
- EMA Truqap product information, section 5.1

Source reconciled; independent clinical, pharmacy and patient/PPI review pending